



# MICHIGAN HEALTH EDUCATION STANDARDS GUIDELINES



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# Michigan Health Education Standards Guidelines

## Vision

The goal of health education is to develop an individual who is health-literate and engages in practices that lead to an overall healthy lifestyle throughout their life. Best practice in K-12 education includes comprehensive health education in all grades, focusing heavily on skill development.

All Michigan students will learn how to take care of their health through clear, accurate, and age-appropriate lessons. These lessons will help students build healthy habits, understand their emotions, and support their mental well-being. Students will gain the confidence and skills they need to make good choices and live healthy lives.

## Rationale and Updated Focus

Michigan's 2007 Health Education Content Expectations reflected legal requirements, best practices, and current research at the time in the teaching and learning of health education. They built on the Michigan Health Education Standards and Benchmarks (1996) and the State Board of Education's Policy on Comprehensive School Health Education (2004). This updated version is more robust and adds details and attention to the critical skills of comprehensive health education, and makes links among students' well-being, school performance, and family involvement.

These Michigan Health Education Standards Guidelines are built upon the National Health Education Standards drafted, reviewed, and passed in March 2024 by the Society of Health and Physical Educators (SHAPE) America and modeled, with permission, on the Massachusetts Curriculum Guidelines for Comprehensive Health and Physical Education (2023). Not only does Massachusetts rank number one in its math and reading scores according to the 2024 National Assessment of Educational Progress, but it also has lower student rates of bullying, depression, and suicide. These Michigan Health Education Standards Guidelines were modified to align with Michigan laws.

Health education programs are a critical component of a well-rounded education that helps support the whole child. As the world continues to experience complex health challenges, a comprehensive approach to support the development of healthy students remains a key need today and in the future. These standards guidelines provide a pathway for and continue to encourage all schools to implement educational programming and strategies that enhance students' mental, emotional, and physical health while recognizing the critical role of school and community climate and culture on student outcomes. The focus is on developing self-efficacy (the belief in one's own ability to successfully perform a specific task or handle a particular situation) in health-related skills and knowledge at the student

level, and implementation of policies and practices at the school and district levels.

This involves all staff working with parents to address student well-being, such as building social and emotional competencies or managing stress, as well as teaching skills and information necessary for students to strive to improve health outcomes and health-related behaviors for themselves and their communities. Staff most directly involved in comprehensive health education programs include health educators, as well as school nurses, school psychologists, and counselors, among others.

The updated focus of the 2025 standards guidelines reflects several key developments and shifts in the field of health education to support student success. These shifts include:

- A stronger emphasis on practices – the processes and skills needed to promote and maintain lifelong physical, mental, and emotional health. Practices are emphasized both in the overall organization of the standards guidelines as well as in individual standards guidelines, resulting in more active and rigorous performance expectations that reflect real-world applications.
- A clear articulation of social and emotional competencies that can apply across the curriculum and be integrated into any area of content.
- Updates to reflect current and ever-changing technology, social media, and other influences on health and well-being (e.g., vaping, opioid use, health during a crisis). While specific references are kept somewhat generic because each can change quickly, the standards guidelines emphasize critical analysis and strategies to evaluate health issues and their potential effect on individuals and the community.
- Updates to acknowledge that students are increasingly using media and technology to access information and services related to health and health care and emphasize the importance of including media literacy as a component of health and well-being.
- A more deliberate integration of skills for personal safety, maintaining personal boundaries, and child sexual abuse prevention through a trauma-informed lens. The guidelines are meant to help students understand that abuse is never their fault, and that trauma is something that people may experience, but it does not define who they are.
- A trauma-responsive perspective that supports and encourages students in their learning while recognizing that students bring individual and unique needs to the classroom.
- Guiding principles that encourage a whole school, whole community, whole child approach and emphasize the application of the *practices* noted below across disciplines, throughout school programming, and coordinated planning across the curriculum and the general school environment.

- In this document, the term “developmentally appropriate” recognizes that development is comprised of physical, cognitive, and social and emotional domains. These guidelines do not define the additional resources and supplemental supports that may be necessary to meet varying developmental needs. This includes designing and implementing learning experiences and opportunities that recognize both the internal assets students bring into the classroom and the external and social forces outside of students’ control that may affect learning. It is up to administrators, educators, and parents to provide all students with the opportunity to learn and meet the same high standards needed to access the skills and knowledge that will be necessary in their lives.

## Standards Guidelines vs. Curriculum

[MCL 380.1278](#) assigns the State Board of Education (SBE) the responsibility to develop and periodically update model core academic curriculum content standards. [MCL 388.994](#) states that the SBE provides its constitutional general supervision by developing and creating general guidelines and standards by which public education is provided. These standards are educational goals that define what students should know and be able to do by the end of each grade span. Standards typically do not provide all the details, particular skills, or the “how” of teaching, but are broad in scope and reach to ensure that they support all of Michigan’s 1.36 million students.

Michigan is a local control state. Therefore, local education agencies (LEAs) use standards and guidelines set by the State Board of Education to select a curriculum that best supports the needs of their students. A curriculum is a program created or selected by a local school district that outlines the content and methods of instruction, including activities, lessons, materials, and assessments, which align with state standards and guidelines.

In summary, academic content standards define what all students are expected to know and be able to do, not how local teachers teach. The standards and guidelines focus on what is most essential for student learning rather than describing all that can or should be taught.

The Michigan Health Education Standards Guidelines meet the requirements of [MCL 380.1278a](#) (referred to as “standards guidelines” throughout the rest of the document). These standards guidelines include the key practices and concepts that students broadly need to be healthy in school and life. The standards guidelines typically do not provide all the details, particular skills, or functional information that may be part of a curriculum chosen at the local level. In Michigan, school districts have discretion to determine how the standards guidelines will be implemented at the local level, within the parameters of Michigan law.

## Guiding Principles for Effective Health Education Programs

The guiding principles inform the selection and evaluation of health education curricula that are effective, inclusive, medically accurate, developmentally appropriate, age-appropriate, and include trauma-informed strategies. Curricula guided by these principles will prepare students for postsecondary education, career and technical education, and their lives as productive and informed people in a global society.

### Guiding Principle 1

Students come from a variety of backgrounds. Partnering with educators, families, and community stakeholders reflecting students' backgrounds provides essential support for the implementation of a successful health education program, in which all are invested in supporting students' personal health and the overall health of their community.

### Guiding Principle 2

Effective health education programs provide safe and supportive learning environments that are developmentally appropriate, age-appropriate, trauma-responsive, inclusive, and culturally responsive so that all students, regardless of ability or circumstance, are supported as individuals and can achieve their learning goals.

### Guiding Principle 3

Effective health education programs incorporate diverse perspectives and acknowledge that attainment of equitable and optimal health is contextual, individualized, and affected by the intersection of many factors.

### Guiding Principle 4

Every student deserves equitable access to effective health education programming, including dedicated instruction for health education every year from kindergarten through grade 12, facilitated by qualified educators who are properly endorsed and certified.

### Guiding Principle 5

Effective health education programs foster equity-focused and trauma-informed strategies through school-wide and community collaboration. They support and promote a sense of belonging and well-being and the development of social and emotional skills, including self-awareness, self-management, social awareness, responsible decision-making, and relationship skills, in a wide variety of contexts and situations.

## Guiding Principle 6

Health education programs use various effective implementation and assessment strategies such as scaffolding, collaboration, application, relevance, authenticity, differentiation and adaptation, and authentic performance-based assessment, which provide multiple opportunities for learning and demonstrating competency.

## Guiding Principle 7

Health education programs develop students' skills in research, reasoning, decision-making, critical thinking, problem-solving, and habits of mind necessary for maintaining health throughout their lifespan. These include being able to differentiate among various factors affecting behavior, such as culture, community, family, peers, and group dynamics.

## Guiding Principle 8

Health education programs require a school-wide culture that promotes health and well-being, equity, integration, and collaboration among education leaders and health professionals and coherent district-wide support for implementation to improve each student's cognitive, physical, social, and emotional development.

## Practices for Comprehensive Health Education

The kindergarten through grade 12 practices for health education are the processes and skills that students will learn throughout the elementary, middle, and high school years that promote and maintain lifelong health and well-being. These practices support the development of skills that students need throughout life and are reinforced and applied across disciplines and settings. This approach is evidence-based and aligns with best practices outlined in the "Characteristics of an Effective Health Education Curriculum" from the Centers for Disease Control and Prevention and the "Essential Components of Health Education" from the Society of Health and Physical Educators (SHAPE) America.

The practices closely align with the skills in the National Health Education Standards (NHES). The first standard, focused on functional health information, is now integrated into each of the practices. These practices are so important to health education outcomes that they provide the organization for these guidelines. Their use in this way provides for active and rigorous performance expectations that reflect real-world application.

The practices outline opportunities for students to demonstrate behaviors that support health and well-being and increase health outcomes through the development of self-efficacy, health literacy, and physical literacy. The application of each practice to specific topics is not exhaustive but is intended to outline the progression of the practice across topics and grade spans. Designers of curricula, assessments, and professional development can consider the transferability of these practice skills across topics and through multiple disciplines beyond those covered in this document.

**Practice 1: Self-Awareness and Analyzing Influences:** Examine how emotions, thoughts, needs, values, beliefs, and other factors (both internal and external) influence behaviors, and articulate how these influences affect health behaviors and outcomes.

Through developmentally appropriate and age-appropriate means, students can develop a sense of self-awareness, recognizing their own emotions and needs. They can articulate these feelings and needs to others, expressing them in developmentally appropriate and culturally sensitive ways. As students progress, their physiological vocabulary expands, enabling them to describe the sensations and experiences in their bodies. Similarly, their emotional vocabulary grows, allowing them to articulate their feelings more accurately and effectively. This heightened self-awareness fosters empathy, as students can apply their knowledge of their own emotions to infer the experiences of others and respond with compassion.

Students recognize the various influences around them and critically evaluate these influences to understand how they enhance or inhibit well-being. Students understand that influences may include the culture in which they are immersed, economic systems, peers and families, media, and technology. Older students evaluate their own health-related beliefs and values, discern the sources of those values, and determine whether their personal choices and goals align with their identified values and beliefs, which build critical health and physical literacy. Students consider how individual actions intersect with personal and community health and well-being, and ways that external factors shape our environment and our health.

**Practice 2: Social Awareness, Relationship, and Communication**

**Skills:** Enhance relationships, personal health, and the health of others through social awareness and effective communication.

Students are socially aware individuals who recognize the complexities of the world around them, including the role of health in individual and community success and outcomes. Students understand the interconnectedness of health, how others' health affects individuals' health, and vice versa. They can take the perspective of and empathize with others, including those from diverse backgrounds and cultures. They seek to better understand others and their perspectives. They treat all individuals with respect and apply



strategies to meaningfully engage with family, school, and community resources and supports.

Students demonstrate positive relationship skills. They establish and maintain meaningful and rewarding relationships with diverse individuals and groups. They use verbal and non-verbal skills to develop and maintain healthy personal relationships and ask for help to enhance the health of self and others.

In developmentally appropriate and age-appropriate ways, students communicate clearly and effectively (with considerations for ability and culture) in a variety of settings, situations, and cultural contexts. They cooperate with others, recognize and navigate complex group dynamics, resist inappropriate social pressure, constructively negotiate conflict, and respectfully and assertively communicate needs, wants, and feelings to support their health and avoid problems. This may include telling a parent or other trusted adult if they feel threatened or harmed and employing refusal, negotiation, and collaboration skills to enhance their own health and reduce health risks. Students develop the ability to extract information from a variety of forms of communication and apply it in new settings and circumstances.

**Practice 3: Information and Resource Seeking:** Access, evaluate, and use valid and reliable health information, products, services, and related resources.

Students can seek developmentally appropriate and age-appropriate information from reliable sources to enhance health and well-being. For younger students, information and resources may come from an educator, school nurse, parent, family member, or an adult at home. As students grow and seek information from new sources (e.g., online, healthcare providers, community resources, social media), they must apply increasing sophistication to evaluate the level of expertise, credibility, and potential bias of their sources to use this information in meaningful ways.

Media literacy is enhanced by students analyzing sources of information and resources to determine the extent to which media information and resources help or hinder health. Students know how to use health products (e.g., toothbrushes, adhesive bandages, deodorant, reproductive health products, wearable technology) in ways appropriate for the given situations. They also determine the accessibility of products and services that enhance health and are appropriate for their given context or situation. Students can seek health services and resources that are culturally relevant and responsive to their needs for themselves and others. For younger students, this may take the form of learning how to call 9-1-1 when someone needs help or how to identify community or school health helpers. In the upper grades, students

may learn to identify and use community resources to support health care needs or develop coping strategies.

**Practice 4: Decision Making and Problem Solving:** Make health-promoting, informed, responsible decisions and solve problems in a variety of health-related situations.

Students can make informed, responsible decisions to lead a life that enhances overall well-being across numerous aspects of health. Students can apply developmental and age-appropriate decision-making processes in situations. Through developing this practice, students increase their health and physical literacy and can make decisions that improve health and well-being in a variety of settings and situations.

Students can examine their options and the potential consequences, consider how personal beliefs and values influence their decisions, the role external forces play on decisions and opportunities, and evaluate the results of their decisions.

In developmentally appropriate and age-appropriate ways, and through a variety of health-related situations, students can apply a decision-making model to evaluate the benefits and risks of various alternatives when addressing problems. Students can differentiate between a decision that can be made individually and a decision that may need assistance. Students can work collaboratively in various settings and groups to solve problems while navigating group dynamics.

**Practice 5: Self-Management and Goal Setting:** Set goals, engage in health-promoting behaviors, and avoid risky behaviors.

In developmentally appropriate and age-appropriate ways, students can recognize and regulate their emotions, actions, and behaviors in different situations to effectively manage stress, control impulses, and self-motivate. Students use health-promoting strategies (e.g., physical activity, asking for help) and avoid risk behaviors (e.g., using food or alcohol to cope) to meet their social, emotional, and physical needs. Students take personal responsibility for their health, while recognizing factors outside of their control that can affect health and well-being. Students will seek support to navigate health challenges. They engage in health-promoting behaviors in a variety of settings and can explain how these behaviors contribute to a positive quality of life and prevent injury and disease.

Students must become self-aware through self-reflection and examination of their identities, lives, habits, and behaviors to identify, adopt, and maintain health-promoting behaviors and lifestyles. They set short-term and long-term goals that are specific, measurable, attainable, relevant, and time-bound. The process often includes identifying who can help (e.g., a health education teacher, parent, guardian, family member, and/or community

organization) when assistance is needed to set and achieve a personal health goal. For older students, the process may include describing how personal health goals can vary with changing abilities, priorities, opportunities, and responsibilities.

**Practice 6: Advocacy and Health Promotion:** Promote personal, family, and community health and well-being.

In developmentally appropriate and age-appropriate ways, students can recognize their own and others' health needs (e.g., physical activity, health care, clean air) and act effectively to address those needs on their own. Students encourage others to embrace similar behaviors and support their efforts with a variety of valid and reliable resources. Students recognize when and how to advocate for health needs for themselves, their family, and their community.

At the personal and family levels, students develop an awareness of what they and their family members need to be healthy. Students will be able to communicate those needs within their family to promote health, raise awareness, seek assistance, and promote change. At the community level, students examine public health issues and engage in relevant advocacy efforts to promote health and well-being.

For younger students, advocacy and health promotion may include promoting handwashing or sneezing into one's sleeve to prevent the spread of germs or encouraging students to increase their physical activity to recommended levels. For older students, advocacy and health promotion may include working to reduce the stigma of mental health conditions and helping others to understand the warning signs of suicide. Students focus on promoting well-being for health issues that are meaningful and relevant to them. They will also encourage, influence, and support peers to make positive health-related choices.

## Organization of the Standards Guidelines

The standards guidelines document is organized first by sections:

- Section 1 is content required by state law and includes HIV, cardiopulmonary resuscitation (CPR)/automated external defibrillation (AED), and physiology and hygiene as it relates to substance use;
- Section 2 is the bulk of the general health education standards guidelines; and
- Section 3 is sex education standards guidelines with specific laws governing district responsibilities and parent choice.

There are appendices with all Michigan laws governing health and sex education.

Within each section, content standards guidelines are organized by grade span: K-2, 3-5, 6-8, and 9-12. The change from individual grade levels to grade spans allows more flexibility for districts. The indicators are considered learning goals, which are intended to be achieved by the end of each grade span, respectively. Within each grade span, the indicators are first grouped by practice. This reflects the importance of the practices for students across the discipline of health education and in developing social and emotional competencies.

Within each practice, the indicators are grouped by the following topics:

- Balanced Eating and Physical Activity [BEPA]
- Community and Environmental Health [CEH]
- Healthy Relationships [HR]
- Mental and Emotional Health [MEH]
- Personal Health and Wellness [PHW]
- Safety [SAF]
- Substance Use and Misuse [SU]
- Sex Education [SE]

Below is an example that labels the various components included in the Standards Guidelines:

**Grades K–2** (Grade span meaning by the end of Grade 2)

**Practice 4: Decision-Making and Problem Solving**

*Make health-promoting, informed, responsible decisions and solve problems in a variety of health-related situations.* (Definition of the Practice)

Balanced Eating and Physical Activity [2.4.BEPA] (Topic)

1. Identify and describe strengths and interests related to physical activity.

(Indicator)

## Design of Guidelines

Each practice provides a set of indicators that support developing health literacy and social and emotional competencies. Each grouping of indicators is designed to advance a thoughtful progression of the associated practice and topic. From early elementary grades through high school, the standards guidelines build over time so that students have the foundations necessary for successful engagement and learning of the standards guidelines in later grades. At each grade span, this document aligns practices with topics that are meaningful to explore and develop with students. However, within this document, it is not possible to connect every topic with every practice that may be meaningful for students. Educators may find it appropriate to add topic criteria for a practice to provide students with additional opportunities

to develop skill proficiency. While the progression builds across successive grade spans (i.e., from K-2 to 3-5 to 6-8 to 9-12) for each practice and topic, within any specific grade span, there is no implied sequencing for curriculum and instruction based on the order of the indicators.

Each indicator includes two key components: a performance element (the verb[s]) that begins a standard, and a concept and knowledge element (what is to be learned and applied). Together, these make up a performance expectation that students can demonstrate because of instruction. It is important to note that curriculum and instruction are not limited to the guidelines as written; health educators can mix and match practices, concepts, and performance expectations as needed for their context.

Each indicator has a unique identifying code. Each code indicates the grade span, practice, topic, and standard number. In the example above, the grades K–2 indicator presented is coded as 2.4.BEPA.



## SECTION 1. CONTENT REQUIRED BY STATE LAW

## Introduction

The Michigan Legislature has passed laws that require local districts to teach specific content. This requirement currently includes communicable diseases such as HIV/AIDS under [MCL 380.1169](#), cardiopulmonary resuscitation (CPR) and automated external defibrillators (AED) under [MCL 380.1170a](#), and physiology and hygiene with special reference to substance abuse, including the abusive use of tobacco, alcohol, and drugs, and their effect upon the human system under [MCL 380.1170](#).

It's important for local districts and educators to know and understand that the HIV/AIDS instruction is still subject to the laws around educator qualifications under [MCL 380.1169](#) and parent choice outlined in [MCL 380.1170](#).

CPR standards guidelines fall under the safety topic, and physiology and hygiene, with reference to substance abuse, which falls under substance use and misuse. Both are typically included as part of a local district's choice in the curriculum adopted at the local level.

## Educator Qualifications for Teaching Health Education

[MCL 380.1531](#) gives responsibility to the superintendent of public instruction to determine requirements for and issue all licenses, certificates, and endorsements for teachers. [MDE's Quick Reference Guide: Courses That Can Be Taught](#) outlines endorsements that meet the requirements to teach health education.

A teacher with an elementary certificate (grades K-5) with an endorsement that includes "all subjects" is qualified to teach health education for the grades designated. Although the law allows elementary teachers to teach health education, it is highly recommended by the Michigan Department of Education that teachers receive professional development from an approved program in health and sex education, which includes training in curriculum implementation, best practice, and current state law related to health education. Please contact your [Regional School Health Coordinator](#) to inquire about current training opportunities.

A teacher with a secondary certificate (grades 6-12) is authorized to teach health education with any one of the following endorsements: MC (Health and Physical Education), *MA (health)*, *MC (K-12 Health and Physical Education)*, *MX (health, physical education, recreation, and dance)*, and KH (family and consumer science). Please note that the italicized endorsements are no longer issued and are in the process of being phased out.

## Educator Qualifications for Sex Education and HIV/AIDS Instruction

Pursuant to [MCL 380.1507](#), in order to teach sex education (puberty) at the elementary level, a teacher must be qualified to teach health education. See designations in the section above, titled “Educator Qualifications for Teaching Health Education.”

To teach sex education at the middle and high school level under a secondary teaching certificate, teachers must be endorsed to teach health under [MCL 380.1507\(5\)](#). See above for the endorsements that qualify teachers to teach health. Due to the restrictions on who can legally teach sex education, at the secondary level, sex education is traditionally taught as a stand-alone unit within the health education course. **Keep in mind that the inclusion of the sex education unit within the health course does not mean that sex education is required to be taught to students for them to receive their health education credit for graduation.** [MCL 380.1507](#) clearly states in Section 1507(2) that the class (sex education) described in subsection (1) shall be elective and not a requirement for graduation.

Qualifications of a person who may provide HIV/AIDS instruction:

- Consistent with [MCL 380.1169](#) that mandates HIV/AIDS instruction, any certified teacher shall be qualified to provide such instruction upon the successful completion of an in-service program provided by designated [Regional School Health Coordinators](#) and approved by the Michigan Department of Education.
- Apart from certified teachers, [MCL 380.1169\(2\)](#) of the Revised School Code also allows licensed health care professionals who have training in HIV/AIDS to provide this instruction.

## Parent Choice in Sex Education and HIV/AIDS Instruction

By law ([MCL 380.1507](#)), parents shall:

- Receive prior notification of sex education classes and curriculum.
- Have the right to review sex education curriculum prior to instruction.
- Be able to opt out their child from all or some of the sex education content without penalty.

The parent rights listed above also pertain to HIV instruction under [MCL 380.1169](#).

Michigan law [MCL 380.1507](#) has some of the strongest requirements in the country for parental involvement and oversight in sex education. If a district chooses to include sex education as part of its comprehensive health education program, the district must establish a sex education advisory



board. This board (SEAB) must consist of at least 50% parent membership. The majority of the parents cannot currently be employed by the school district. The SEAB must also include at least one parent as a co-chair. SEABs review student data, review curricula, and make recommendations to the local district's board of education for review and approval.

Local education agencies (LEAs), including intermediate school districts, public school academies, and traditional public school districts, have local control in determining whether to include sex education as a unit in their health education course. However, the required topics described in this section are not optional.

## Requirements of Michigan Laws

**Physiology/Hygiene:** [380.1170](#) - Physiology and hygiene; instruction; development of comprehensive health education programs; conflict with religious beliefs.

**Physiology/Hygiene Parent Opt Out:** [380.1170](#) - Section 3: A child, upon the written statement of parent or guardian that instruction in the characteristics or symptoms of disease is in conflict with his or her sincerely held religious beliefs, shall be excused from attending classes where such instruction is being given and no penalties as to credit or graduation shall result therefrom.

**Dangerous Communicable Diseases, including HIV:** [380.1169](#) - Dangerous communicable diseases; human immunodeficiency virus infection and acquired immunodeficiency virus infection; teacher training; teaching materials; curricula; teaching of abstinence from sex.

**Dangerous Communicable Diseases, including HIV Parent Opt Out:** [380.1170](#) - Section 3: A child, upon the written statement of parent or guardian that instruction in the characteristics or symptoms of disease is in conflict with his or her sincerely held religious beliefs, shall be excused from attending classes where such instruction is being given and no penalties as to credit or graduation shall result therefrom.

**CPR:** [380.1170a](#) - Model core academic curriculum content standards, guidelines for health education; subject area content expectations and guidelines for health education; instruction in cardiopulmonary resuscitation and automated external defibrillators; individuals providing instruction; use of local resources; exemption; definitions.

## Section 1: Standards Guidelines That Are Required by Law to Be Taught

Please note that the following indicators, listed under this section, are repeated within their identified health topics in Section 2 (safety and substance use and misuse) and Section 3 (sex education). These are pulled out here to show clearly which content must be taught by Michigan laws noted above.

**Please remember that while the sex education topics addressing dangerous communicable diseases in this section must be taught, parent choice outlined in [MCL 380.1170](#) remains.**

### Grade Span: K-2 (by the end of Grade 2)

Purposely Blank

There are NO standards guidelines in this grade span.

## Grade Span: 3-5 (by the end of Grade 5)

### **Practice 2:** Social Awareness, Relationship, and Communication Skills

*Enhance relationships, personal health, and the health of others through social awareness and effective communication.*

#### *Substance Use and Misuse [5.2.SU]*

1. Distinguish between the use and misuse of drugs (legal and illegal) and identify potential short- and long-term effects on the body.

### **Practice 3:** Information and Resource Seeking

*Access, evaluate, and use valid and reliable health information, products, services, and related resources.*

#### *Sex Education [5.3.SE]*

1. Define communicable diseases, including Human Immunodeficiency Virus (HIV), and identify how they are and are not transmitted.

## Grade Span: 6-8 (by the end of Grade 8)

### **Practice 1: Self-Awareness and Analyzing Influences**

*Examine how emotions, thoughts, needs, values, beliefs, and other factors (both internal and external) influence behaviors and articulate how these influences affect health behavior and outcomes.*

#### *Substance Use and Misuse [8.1.SU]*

1. Summarize and communicate the effects of using legal (e.g., prescription drugs prescribed to you, over-the-counter drugs, and [at a certain age] nicotine, electronic vapor products, alcohol and marijuana) and illegal drugs (e.g., prescription drugs not prescribed to you, cocaine) on brain development and multiple dimensions of health (e.g., physical, social, occupational, mental/emotional).

### **Practice 2: Social Awareness, Relationship, and Communication Skills**

#### *Sex Education [8.2.SE]*

1. Discuss signs, symptoms, and potential effects of sexually transmitted infections, including HIV.

### **Practice 4: Decision Making and Problem Solving**

*Make health-promoting, informed, responsible decisions and solve problems in a variety of health-related situations.*

#### *Sex Education [8.4.SE]*

1. Analyze ways to prevent sexually transmitted infections (STIs), including strategies that can be used before becoming sexually active (e.g., communicating with a partner, abstinence).
2. Articulate the benefits of abstinence, postponing sexual activity, and setting personal limits (e.g., aligning with personal or family values, changing the nature of relationships, reducing risk of STIs) based on individual beliefs and values.

### **Practice 5: Self-Management and Goal Setting**

*Set goals, engage in health-promoting behaviors, and avoid risky behaviors.*

#### *Safety [8.5.SAF]*

1. Demonstrate proficiency in cardiopulmonary resuscitation (CPR) and the use of automated external defibrillators (AEDs).

### *Sex Education [8.5.SE]*

1. Determine strategies, including abstinence, that will reduce the risk of HIV and other sexually transmitted infections.

## Grade Span: 9-12 (by the end of Grade 12)

### **Practice 3: Information and Resource Seeking**

*Access, evaluate, and use valid and reliable health information, products, services, and related resources.*

#### *Substance Use and Misuse [12.3.SU]*

1. Use valid and reliable information to analyze the relationship between the use of legal and illegal (regarding legal age of use and type) substances and the causes of death (including overdose) and disease (including addiction) in the United States.

### **Practice 4: Decision Making and Problem Solving**

*Make health-promoting, informed, responsible decisions and solve problems in a variety of health-related situations.*

#### *Substance Use and Misuse [12.4.SU]*

1. Analyze the potential short- and long-term impacts of legal and illegal substances on multiple dimensions of health (e.g., physical, mental, emotional, social, and/or intellectual) and on other health risk behaviors (e.g., sexual activity and impaired driving).

#### *Sex Education [12.4.SE]*

1. Analyze factors that contribute to behaviors that increase the risk of HIV and other STIs.
2. Explain the importance of STI (including HIV) testing and treatment, where to get tested, and why it is essential to communicate with a partner about STI status.
3. Identify situations, signs, and symptoms that might indicate a need to seek medical consultation.
4. Analyze factors that contribute to behaviors that increase the risk of HIV and other STIs.

### **Practice 5: Self-Management and Goal Setting**

*Set goals, engage in health-promoting behaviors, and avoid risky behaviors.*

#### *Safety [12.5.SAF]*

1. Demonstrate proficiency in cardiopulmonary resuscitation (CPR) and the use of automated external defibrillators (AEDs).



## SECTION 2. GENERAL HEALTH EDUCATION CONTENT

## Introduction

These standards guidelines in Section 2 serve as guidance to local districts as they choose their health curriculum for any grade, K-12. Not all topics may be covered; it is a local decision based on local student data which topics will be covered and at what grade span(s).

## Best Practices in Health Education Instruction

With the limitations of time to teach about all the health issues affecting students' lives, it is important to prioritize curriculum and instruction around the most significant health issues affecting students' lives within each grade span. Regularly reviewing risk and protective factor data, along with risk behavior data concerning students, can help immensely with prioritizing instruction. Each group of students that comes through a teacher's classroom may be dealing with very different risk factors than the students before them, even just a year apart.

Best practice for high school health instruction is for health to be taught during grades 11-12, when high school content is more relevant to the lives of the students and will set them up for success into adulthood. A high school health course is required for graduation under [MCL 380.1278a](#). Please note that a sex education unit within a health course is NOT required as part of the graduation requirement as stated in [MCL 380.1507 Sections 1 and 2](#).

This guidance also provides best practices that are included in Michigan law but are not required to be covered in relevant health topics, including, but not limited to:

- Health and physical education for students shall be established and provided in all public schools in the state ([MCL 380.1502](#) Building Level Health Education)
- Bullying Prevention that addresses preventing, identifying, responding to, and reporting incidents of bullying and cyberbullying (Matt Epling's Law – [MCL 380.1310b](#))
- Child Sexual Abuse Prevention that includes evidence-based instruction for students in pre-K-5 (Erin's Law – [MCL 380.1505](#) and [MCL 380.1505a](#))
- Depression and Suicide Awareness (Chase Edwards Law – [MCL 380.1171](#))
- Prescription Pills and Opioid Prevention ([MCL 380.1170b](#))

## Michigan Laws Specific to Health Education Instruction

According to [MCL 380.1502](#), Michigan requires all public schools to establish and provide health education to students. At the K-8 level, the law does not dictate how much time schools must spend on health education content. This allows districts to determine the needs of their students, the time they can dedicate to the content, and the length of the program they offer. Knowing that schools' implementation of health education will vary greatly based on



these factors and others, these comprehensive guidelines allow districts to select the skills and topics that will have the greatest impact on their students' health and well-being.

While health education is a requirement for graduation, districts can select the course content, methods, and instructional resources that will meet that requirement. If a district chooses to include a sex education unit, or any sex education standard, as part of the health education course, parents still have the right to opt their child out of sex education without penalty or loss of academic credit ([MCL 380.1507](#)).

## Educator Qualifications for Teaching Health Education

A teacher with an elementary certificate (grades K-5) with an endorsement that includes "all subjects" is qualified to teach health education for the grades designated. Although the law allows elementary teachers to teach health education, it is highly recommended by the Michigan Department of Education that teachers receive professional development from an approved program in health and sex education, which includes training in curriculum implementation, best practice, and current state law related to health education. Please contact your [Regional School Health Coordinator](#) to inquire about current training opportunities.

A teacher with a secondary certificate (grades 6-12) is authorized to teach health education with any one of the following endorsements: MC (Health and Physical Education), *MA (health)*, *MC (K-12 Health and Physical Education)*, *MX (health, physical education, recreation, and dance)*, and KH (family and consumer science). Please note that the italicized endorsements are no longer issued and are in the process of being phased out. For more information regarding whether your certification allows you to teach health, please go to the Michigan Department of Education's [Quick Reference Guide: Courses That Can Be Taught page](#).

# Section 2: Health Education Standards Guidelines

## Grade Span K–2 (by the end of Grade 2)

### **Practice 1: Self-Awareness and Analyzing Influences**

*Examine how emotions, thoughts, needs, values, beliefs, and other factors (both internal and external) influence behaviors and articulate how these influences affect health behavior and outcomes.*

#### ***Mental and Emotional Health [2.1.MEH]***

1. Recognize and accurately identify basic emotions (e.g., happy, sad, mad, worried, lonely).
2. Explain that emotions are information and that one's emotions may be the same or different from the emotions of others.
3. List and demonstrate emotional regulation strategies to support mental and emotional health independently or with support.
4. Recognize and list challenges and setbacks as a regular part of life.
5. Demonstrate the ability to persevere despite perceived challenges and setbacks.
6. Describe personal strengths and the ways that those strengths support mental health.
7. Articulate and celebrate the individual characteristics that make a person unique and explain that different factors (e.g., peers, media, culture, family, phase of life, etc.) can influence how a person views themselves.
8. Demonstrate strategies that help all students feel welcome and valued as a part of the school community (e.g., cooperative playing, listening, showing you care, sharing).

### **Practice 2: Social Awareness, Relationship, and Communication Skills**

*Enhance relationships, personal health, and the health of others through social awareness and effective communication.*

#### ***Mental and Emotional Health [2.2.MEH]***

1. Identify and practice strategies to make and keep friends and develop positive peer relationships (e.g., identify and acknowledge other people's feelings, communicate effectively, ask for help).
2. Identify reasons why it is important to have a variety of supportive relationships.
3. Identify characteristics of a trusted adult.
4. Identify feelings and practice talking about them to

- parents/caregivers, family members, and/or trusted adults.
5. Express needs, wants, and feelings through verbal and nonverbal actions.
  6. Show respect for the feelings, rights, and property of others.
  7. Demonstrate effective listening and communication skills, including giving and accepting compliments and feedback, individually and in groups.
  8. Recognize and appreciate individual differences in others.
  9. Describe positive qualities in self and others.
  10. Identify reasons for disagreements and/or conflict and identify strategies for resolving and/or managing them.

### *Healthy Relationships [2.2.HR]*

1. Define bullying and teasing, explain their similarities and differences, and how both can be harmful.
2. Define personal boundaries and demonstrate simple ways to communicate them. This includes respecting the boundaries of others, including physical, verbal, and emotional boundaries.
3. Explain how no one has a right to violate personal boundaries and demonstrate an appropriate refusal (e.g., tell a trusted adult, say NO, leave the situation) when someone says or does something that does not respect personal boundaries.
4. Identify groups to which one belongs and reflect on similarities and differences with others.
5. Recognize the benefits of and strategies for cooperation in various settings.
6. Discuss stereotypes and how these can affect relationships and situations.
7. Appreciate and demonstrate empathy and respect for others.
8. Demonstrate awareness of and respect for a variety of family structures.
9. Anticipate how someone else may feel in various situations and display compassionate and empathetic behaviors.

### *Balanced Eating and Physical Activity [2.2.BEPA]*

1. Invite others of various ability levels to join and participate in physical activities.

## **Practice 3: Information and Resource Seeking**

*Access, evaluate, and use valid and reliable health information, products, services, and related resources.*

### *Substance Use and Misuse [2.3.SU]*

1. Recognize that medication comes in many forms and always seek assistance from a parent/caregiver or other trusted adult.
2. Explain the importance of using prescription and over-the-counter

- medications correctly and safely and the potential risks associated with misusing and/or ingesting household products.
3. Identify trusted adults at home and school who can discuss rules and practices related to medicine use (e.g., only taking medicine with an adult's help, only taking prescriptions prescribed to you).

### *Personal Health and Wellness (2.3.PHW)*

1. Identify school and community health worker resources.
2. Identify individuals who can assist with health-related issues and potentially life-threatening health conditions (e.g., asthma episodes, diabetes, allergic reactions, seizures, concussions).
3. Demonstrate the ability to access help for self or others to support personal health and wellness.

## **Practice 4: Decision Making and Problem Solving**

*Make health-promoting, informed, responsible decisions and solve problems in a variety of health-related situations.*

### *Balanced Eating and Physical Activity [2.4.BEPA]*

1. Identify situations when a food-related decision needs to be made (e.g., when trying new foods, choosing snacks and beverages, eating breakfast).
2. Identify, with adult guidance, food-related decisions that can be made independently (e.g., asking for healthier options) or when assistance is needed (e.g., managing a food allergy).
3. Describe how food provides nutrients and energy for the body and identify foods that are good sources of energy and nutrients to support informed decision-making.
4. Explain the importance of access to fresh and affordable food and clean drinking water on health.
5. Explain the benefits of health-promoting eating choices and habits (e.g., eating a variety of foods, staying hydrated, awareness of hunger and thirst signals).
6. Describe how the foods students eat may reflect the area in which they live and/or their cultural backgrounds, the way students' families use or produce food, how family meals and food traditions benefit them, different dietary needs (e.g., food allergies, dietary restrictions), and how they contribute to food-related decisions.
7. Recognize that media (e.g., cartoons, characters, advertisements, product placement) can affect food-related decisions.
8. Recognize body responses, physiological and emotional, to choices and habits in relation to eating, movement, and physical activity.
9. Identify and describe strengths and interests related to physical activity.
10. Identify physical activity as a health-promoting habit that contributes to overall health and well-being and list the benefits of

- these habits on physical well-being (e.g., activities that strengthen the heart and cardiovascular systems, contribute to fitness and muscle-building) and mental health (e.g., stress management).
11. Identify opportunities, in and out of the school setting, for safe, active play, and physical activity for self-expression, social interaction, personal enjoyment, and challenge.
  12. Set a short-term physical activity goal relevant to specific needs and abilities, take meaningful action toward achieving the goal, and identify people at home or at school who can help when assistance is needed to achieve the goal.

## **Practice 5: Self-Management and Goal Setting**

*Set goals, engage in health-promoting behaviors, and avoid risky behaviors.*

### ***Mental and Emotional Health [2.5.MEH]***

1. Demonstrate self-control (e.g., delay gratification, wait your turn) independently or with the support of adults in various settings (e.g., on the playground, in the classroom, during physical education, at an assembly).
2. Define stress and demonstrate strategies for managing stress (e.g., positive self-talk, belly breathing, talking with a trusted adult, physical activity, listening to calming music, play).
3. Identify what it means to be responsible and list personal responsibilities.
4. Use positive self-talk to help with emotional regulation and demonstrate supportive behaviors.

### ***Safety [2.5.SAF]***

1. Apply strategies for staying safe in various situations (e.g., on the playground, during physical activity, around water, when using wheeled recreation, as a pedestrian, around cooking elements or fire, on the bus, when online, around weapons, or in situations of gun violence) and determine when to report potentially unsafe situations to an adult.
2. Provide examples of how rules can keep children safe and identify rules to help children stay safe in various situations (e.g., related to medicines, playground safety, physical activity, threats of violence, personal space, and boundaries).
3. Demonstrate how to respond (e.g., yell, get away, tell an adult, seek help) and get help in various emergency situations including when and how to call 9-1-1.
4. Identify safe adults to confide in and places to go if feeling personally threatened (e.g., someone says they will hurt or harm you).
5. Demonstrate the ability to ask a trusted adult for help (including

- problem-solving) in various situations.
6. Recognize safe and unsafe touching and demonstrate how to tell a parent, guardian, or other trusted adult if this happens.
  7. Identify and practice behaviors for personal safety (e.g., say no, get away, tell a trusted adult).

### *Personal Health and Wellness [2.5.PHW]*

1. Identify various habits that can promote health (e.g., brushing teeth, proper nutrition, going to the doctor, getting enough sleep, being physically active, limiting screen time, washing hands, etc.).
2. Explain how a young person can maintain or enhance the health of both their body and mind.
3. Use medically accurate names for body parts when communicating about the body and physical health.
4. Demonstrate independence in health-promoting practices such as hand washing, tooth brushing, sneezing and coughing, disposal of tissues, and physical activity.
5. Set a simple goal related to physical health habits and monitor progress toward the goal with assistance from a parent, guardian, or other trusted adult.

## **Practice 6: Advocacy and Health Promotion**

*Promote personal, family, and community health and well-being.*

### *Personal Health and Wellness [2.6.PHW]*

1. Describe personal health habits (e.g., brushing and flossing teeth, hand washing, bathing and/or showering, sufficient sleep, sun safety, physical activity, limiting screen time) that can prevent illness and promote self-care and overall health.
2. Advocate for personal health needs to be met (e.g., needing to wash hands, asking for sun protection, access to bathrooms).
3. Recognize how the actions of others can affect physical health (e.g., spreading germs) and encourage peers to make positive choices about physical health habits and prevention strategies.

### *Community and Environmental Health [2.6.CEH]*

1. Identify the effects of personal activities that positively or negatively contribute to the environment.
2. Identify ways that the communities people live in and connections to others, can affect their health and well-being.
3. Identify strategies to minimize environmental impact (e.g., reduce, reuse, recycle).
4. Encourage peers and family members to make choices to help protect the environment (e.g., recycling, using less water, turning off lights).

5. Encourage peers and family to help in the community (e.g., donate food to a food pantry, clean up litter).

## Grade Span: 3-5 (by the end of Grade 5)

### **Practice 1: Self-Awareness and Analyzing Influences**

*Examine how emotions, thoughts, needs, values, beliefs, and other factors (both internal and external) influence behaviors and articulate how these influences affect health behavior and outcomes.*

#### *Mental and Emotional Health [5.1.MEH]*

1. Describe personal and cultural identities and assets, their importance and value, and explain how they support mental and emotional health.
2. Describe personal interests and the skills needed to pursue those interests in ways that support individual growth.
3. Identify personal strengths and opportunities for growth and improvement in various contexts.
4. Describe how peers, media, family, society, community, and culture can influence ideas about body image and the impact on self-esteem and behaviors.
5. Demonstrate how social media and technology can influence mental and emotional well-being (e.g., stress levels, happiness, mood).

### **Practice 2: Social Awareness, Relationship, and Communication Skills**

*Enhance relationships, personal health, and the health of others through social awareness and effective communication.*

#### *Mental and Emotional Health [5.2.MEH]*

1. Demonstrate an awareness that emotions may be expressed in different ways (e.g., through body language, intensity of expression) by various groups and in other cultures.
2. Identify people who are trusted adults in various settings.
3. Explain the importance of talking with parents, guardians, family, friends, and/or trusted adults about feelings and emotions.
4. Demonstrate how to ask for assistance with mental health questions, issues, or concerns (e.g., challenges with friends, feeling anxious).
5. Define stigma and demonstrate how to discuss mental health and mental health conditions in ways that reduce stigma.
6. Identify signs and symptoms of mental distress in self and others, and how to get help.

#### *Healthy Relationships [5.2.HR]*

1. Identify characteristics of supportive relationships with a variety of individuals (e.g., family, peers, trusted adults, teachers).
2. Define and demonstrate ways to determine and respect the



- boundaries of self and others.
3. Demonstrate strategies for addressing one's feelings and the feelings and perspectives of others to support positive relationships.
  4. Differentiate between conflict and bullying and articulate the importance of the difference to avoid escalating conflicts into bullying or violence.
  5. Identify and respond to bullying, including practicing supportive bystander behaviors, in various settings.
  6. Identify and practice assertive and nonviolent communication skills.
  7. Identify and practice conflict prevention, management, and resolution strategies.
  8. Describe how personal experiences, peers, family, media, society, community, and culture influence the ways people interact in relationships and social situations.
  9. Describe how stereotypes, perceived stereotypes, and prejudice can affect relationships, and demonstrate strategies to address these factors.

### *Balanced Eating and Physical Activity [5.2.BEPA]*

1. Demonstrate respectful interactions with others when participating in physical activity (e.g., at recess).
2. Identify and describe social benefits gained from participating in physical activity.
3. Identify opportunities and safe places for activity outside of school.
4. Describe the benefits of movement and being physically active, especially with others, on physical, social, and emotional health.

### *Substance Use and Misuse [5.2.SU]*

1. Distinguish between the use and misuse of drugs (legal and illegal) and identify potential short- and long-term effects on the body.
2. Identify potential reasons why people might use legal and illegal drugs, such as media, peer pressure, stress, or cultural factors.
3. Discuss health-promoting strategies to prevent illegal drug use and unsafe or potentially harmful use of illegal drugs.
4. Analyze data about youth substance use to emphasize positive social norms (e.g., most youth are not vaping).
5. Effectively communicate personal feelings or perspectives about substance use and misuse.
6. Demonstrate effective refusal of alcohol, nicotine, electronic vapor products, marijuana, and other substances that can affect health.

## **Practice 3: Information and Resource Seeking**

*Access, evaluate, and use valid and reliable health information, products, services, and related resources.*

### *Safety [5.3.SAF]*

1. Discuss what constitutes abuse, harassment, and assault.
2. Locate trusted adults (including parents or guardians) from whom to get help if boundaries are being violated or one is being abused, harassed, or assaulted.

## **Practice 4: Decision Making and Problem Solving**

*Make health-promoting, informed, responsible decisions and solve problems in a variety of health-related situations.*

### *Balanced Eating and Physical Activity [5.4.BEPA]*

1. Identify principles of balanced eating to meet nutritional needs when making nutrition-related decisions.
2. Identify and describe hunger and fullness indicators and how these can inform nutrition-related decision-making.
3. Discuss the benefits of balanced eating and physical activity on physical health (e.g., supporting growth and development, ability to engage in physical activity), social health, and emotional and mental health (e.g., managing stress and emotions) when making decisions pertaining to nutrition and physical activity.
4. Demonstrate how to use food labels as part of nutrition-related decision-making.
5. Describe how and where food comes from and how food production affects nutrition-related decisions.
6. Identify factors (e.g., budget, food access and availability, time management) that influence decisions about nutrition and determine when assistance is needed to make a decision.
7. Explain various factors that can influence decisions about nutrition (e.g., food during celebrations, food preferences, media advertising, celebrity endorsements, product placement, access and availability, financial resources, physical activity levels).
8. Recognize that individuals have different food-related needs, preferences, and traditions.
9. Use the steps of the decision-making process when making a nutrition-related decision.
10. Engage in independent and cooperative problem-solving activities while participating in physical activities.
11. Compare the benefits of various physical activities to support personal decision-making related to physical activity.

## **Practice 5: Self-Management and Goal Setting**

*Set goals, engage in health-promoting behaviors, and avoid risky behaviors.*

### *Mental and Emotional Health [5.5.MEH]*

1. Identify different feelings and emotions that people may experience

- and how people might express those emotions (including individual and cultural differences in expression).
2. Discuss how feelings and emotions can affect behavior and how behavior can affect feelings and emotions.
  3. Recognize that all feelings and emotions have a purpose and are information that individuals can use to support mental and emotional health.
  4. Identify how a person's brain and body influence mental and emotional well-being.
  5. Describe and demonstrate strategies for expressing and regulating emotions.
  6. Identify characteristics of and practices to support mental and emotional well-being within various cultures and diverse perspectives.
  7. Set a goal to use one or more health-promoting practices or behaviors (e.g., being aware of your own feelings and the feelings of others, safe online behaviors, engaging in physical activity, limiting screen time) and track progress towards its achievement to maintain or improve mental and emotional well-being.
  8. Identify personal stressors and demonstrate effective stress management techniques, independently or with support.
  9. Identify and demonstrate strategies and behaviors to overcome barriers and help meet personal responsibilities.
  10. Identify a variety of strategies for planning, prioritizing, and managing time.
  11. Demonstrate strategies that show a willingness to reflect, learn, and grow from challenges.

### *Safety [5.5.SAF]*

1. Describe ways to promote personal safety and reduce the risk of injuries in various situations (e.g., during physical activity, around motor vehicles, around firearms, around loud noise or music, around water, fire prevention, during a fire, as a pedestrian).
2. Recognize aspects of the environment (e.g., whether crosswalks are marked, presence of sidewalks, weather conditions, access to healthy foods, access to green space, levels of violence in a community) that can positively or negatively affect safety.
3. Identify and demonstrate how to contact appropriate resources when someone is poisoned or injured and needs help (e.g., calling poison control and 911).
4. Apply strategies to stay safe online (e.g., when gaming, using digital technology, and engaging in social media), including addressing overuse.
5. Describe actions one could take if uncomfortable, unsafe, or harmed (e.g., tell a parent, guardian, or other trusted adult, leave the situation).

6. Demonstrate the ability to set and maintain developmentally appropriate boundaries (including physical, verbal, and emotional boundaries) and how to respond if those boundaries are violated.
7. Distinguish between safe, unsafe, and confusing touch and demonstrate strategies for telling a parent, guardian, or other trusted adult.

### *Balanced Eating and Physical Activity [5.5.BEPA]*

1. Demonstrate respect for self and responsible, safe interpersonal behavior (e.g., peer-to-peer, student-to-teacher) that contributes to positive social interaction in various physical activity contexts.
2. Describe the health benefits of regularly participating in physical activity on multiple dimensions of wellness (e.g., stress management, supporting positive mental health, cardiovascular health, fitness levels, muscle strengthening).
3. Recognize how physical activity influences physiological changes in their body.
4. Identify different physical activities for personal enjoyment and challenge, independently and with others.

### *Personal Health and Wellness [5.5.PHW]*

1. Describe personal behaviors and strategies that promote health and/or avoid health risks (e.g., pedestrian safety, sun safety, protecting oneself from infectious diseases, adequate sleep, good nutrition, protective equipment, appropriate screen time, hearing protection, being physically active).
2. Discuss influences on and barriers to maintaining or enhancing physical health and wellness.
3. Set a personal health and wellness goal, identify resources to assist in achieving it, and track progress toward its achievement.

## **Practice 6: Advocacy and Health Promotion**

*Promote personal, family, and community health and well-being.*

### *Community and Environmental Health [5.6.CEH]*

1. Analyze the relationship between personal health and the health of the community.
2. Discuss that people may experience health inequities and health disparities (unfair and avoidable differences in health) due to various factors.
3. Describe ways that the community can influence the health of people within that community.
4. Describe the ways that rules and laws can affect community health and health disparities.
5. Use accurate information when discussing environmental health issues (e.g., littering, deforestation, recycling, clean water) that

- affect people's health.
6. Propose and support classroom policies and behaviors that promote dignity and respect.

## Grade Span: 6-8 (by the end of Grade 8)

### **Practice 1: Self-Awareness and Analyzing Influences**

*Examine how emotions, thoughts, needs, values, beliefs, and other factors (both internal and external) influence behaviors and articulate how these influences affect health behavior and outcomes.*

#### *Mental and Emotional Health [8.1.MEH]*

1. Describe how emotions can affect one's behaviors and experiences, and how this might vary in differing contexts.

#### *Safety [8.1.SAF]*

1. Analyze how various influences (e.g., peers, family, culture, society, school, and community policies) affect adolescents' safety in various situations.
2. Define sexual harassment, sexual abuse, sexual assault, and domestic violence, and identify resources for support.
3. Analyze how sharing or posting personal information electronically about oneself or others (e.g., chat groups, email, texting, sexting, websites, social media, phone and tablet applications) can affect the safety of self and/or others.

#### *Substance Use and Misuse [8.1.SU]*

1. Analyze influences (e.g., culture, peers, media, perceptions of norms) that could lead to the use of nicotine, electronic vapor products, alcohol, or other illegal (regarding age, use, or type) and potentially harmful substances.
2. Describe the ways that social characteristics (e.g., socioeconomic status, culture) can affect risk and protective factors for substance use and misuse.
3. Summarize and communicate the effects of using legal (e.g., prescription drugs prescribed to you, over-the-counter drugs, and [at a certain age] nicotine, electronic vapor products, alcohol and marijuana) and illegal drugs (e.g., prescription drugs not prescribed to you, opioids, cocaine) on brain development and multiple dimensions of health (e.g., physical, social, occupational, mental/emotional).

#### *Community and Environmental Health [8.1.CEH]*

1. Define social factors that affect people's health (e.g., education, social environment, family health history, socioeconomic conditions, food availability, public safety, discrimination) and analyze how they may affect health at different levels (e.g., individual, family, and community).
2. Analyze how stigma and public perception can influence access to

- health and health care (e.g., menstrual health care, mental health services and supports, preventative screenings).
3. Define public health policies and government regulations and explain how they can influence health promotion and disease prevention in both positive and negative ways.
  4. Analyze how environmental factors (e.g., air quality, trash and litter, availability of clean drinking water) and types of pollution (e.g., air, noise, chemical, water) affect health.

## **Practice 2: Social Awareness, Relationship, and Communication Skills**

*Enhance relationships, personal health, and the health of others through social awareness and effective communication.*

### ***Mental and Emotional Health [8.2.MEH]***

1. Discuss how stress and resilience can affect mental and emotional health.
2. Demonstrate ways to support people experiencing stress.
3. Identify how emotions can influence communication (e.g., anger or anxiety may affect the ability to listen well) and demonstrate strategies to communicate effectively when experiencing a range of emotions and in various situations.
4. Accurately recognize and effectively respond to emotions, thoughts, values, and perspectives when communicating with others and resolving interpersonal conflicts.
5. Analyze how people from diverse groups can learn from each other and how this can enhance emotional well-being.
6. Apply refusal or negotiation skills in ways that support or improve mental health and minimize health risks.
7. Advocate for oneself by creating 'I'-statements to express feelings and needs appropriately.
8. Identify signs and symptoms of mental and emotional distress, in self and others, that may require assistance from adults.
9. Demonstrate how to respond (e.g., tell a parent, guardian, or other trusted adult, call or text 9-8-8, OK2SAY tipline) when there is a concern about one's own or someone else's mental well-being or when someone is considering self-harm or suicide.
10. Effectively express needs, wants, emotions, and feelings (including affection, love, friendship, concern, anger) in respectful and health-promoting ways.

### ***Healthy Relationships [8.2.HR]***

(Note: This section should not include anything that falls under sexual relationships or sex education. Sexual relationship topics fall under sex education in these standards guidelines, and must follow state laws on sex education instruction.)

1. Identify characteristics of healthy and unhealthy relationships and ways to seek help in unhealthy or unwanted relationships.
2. Demonstrate effective verbal and non-verbal communication skills that foster healthy relationships, communicate boundaries, and show respect.
3. Articulate how respectful behaviors may vary among populations and how those behaviors contribute to positive social interaction.
4. Explain why respecting a person's boundaries and consent are essential.
5. Demonstrate effective approaches to boundary setting (e.g., acknowledging feelings, communicate the boundary, target alternative) and maintenance of various boundaries (e.g., related to technology use, emotional, physical).
6. Identify situations when boundaries are being violated and identify tactics used to coerce or pressure someone to change a personal boundary (e.g., break a rule, share a password).
7. Demonstrate techniques and assertive responses to counter coercive tactics to maintain boundaries.
8. Analyze how media and technology can positively and negatively influence beliefs about what constitutes a healthy relationship.
9. Analyze the impact of technology and social media on relationships (e.g., use of smart devices, sharing relationship information, location tracking).
10. Describe potential impacts of imbalances in power on a variety of relationships and in various settings.
11. Analyze ways that prejudice, discrimination, and injustice can affect relationship health and describe ways to address these issues to support the health of self and others.
12. Apply conflict resolution strategies in various situations.
13. Demonstrate positive ways to communicate differences of opinion in various relationships (e.g., familial, peer, teacher) and situations (e.g., in class, outside of school, on a team).
14. Differentiate bullying, harassment, and abuse and demonstrate ways to support and seek help for someone who is being bullied, harassed, or abused, or who is the target of unhealthy or coercive behaviors.

### *Substance Use and Misuse [8.2.SU]*

1. Analyze social situations in multiple settings (e.g., at home, at school, out with friends, at a party) that could lead to the use of nicotine, electronic vapor products, alcohol, or other illegal (age, use, or type) and potentially harmful substances.
2. Demonstrate effective verbal and nonverbal communication skills (including refusal) to keep self or others safe in substance use- and misuse-related situations (e.g., avoiding riding with a driver who is



under the influence, resisting peer pressure, seeking help, leaving a situation) to protect individuals from risk or injury.

### **Practice 3: Information and Resource Seeking**

*Access, evaluate, and use valid and reliable health information, products, services, and related resources.*

#### **Safety [8.3.SAF]**

1. Identify sources of support, such as parents, guardians, or other trusted adults, to whom students can go if they or someone they know is being bullied, harassed, abused, assaulted, or exploited.
2. Explain why a person who has been bullied, harassed, abused, or the victim of violence is not at fault.
3. Locate community resources that provide support and resources related to sexual exploitation or for getting help for self or others in situations related to sex trafficking.

#### **Mental and Emotional Health [8.3.MEH]**

1. Describe situations where professional health services are necessary to support or improve mental and emotional well-being.
2. Demonstrate the ability to access professional health services if needed.
3. Locate valid and reliable products, information, and services to enhance mental and emotional well-being, manage stress and emotions, and address mental health conditions.

#### **Personal Health and Wellness [8.3.PHW]**

1. Determine the accessibility of products (e.g., deodorant, hair care, sunscreen, dental care products), resources, and services that enhance health and identify supports and barriers to accessing the products or services.
2. Access personal health products (e.g., deodorant, hair care, sunscreen, dental care products) based on individual needs.
3. Locate various personal health-related digital resources and assess each for reliability and validity.

### **Practice 4: Decision Making and Problem Solving**

*Make health-promoting, informed, responsible decisions and solve problems in a variety of health-related situations.*

#### **Safety [8.4.SAF]**

1. Describe the role of individual versus shared responsibility in staying safe in various situations (e.g., digital safety, threats of violence, dangerous weapons, outdoor recreation, motor vehicle safety).
2. Discuss the variety of systemic, environmental, and physical factors

- that might help or hinder an individual's ability to remain safe in various situations.
3. Evaluate potential options and consequences for decisions related to personal safety (e.g., dangerous weapons, digital safety, threats of violence, motor vehicle safety, physical injury) in a variety of situations.
  4. Describe laws (e.g., age of consent, child abuse and neglect, criminal sexual conduct, minor consent for health care) that relate to young people's health and the rights of adolescents to maintain their own health, and how these might influence decisions related to their health.
  5. Define exploitation, human trafficking (both sex and labor trafficking), and describe strategies used for, and warning signs of, exploitation and recruitment of youth.
  6. Demonstrate strategies perpetrators use to carry out human trafficking and how to get help if concerned about self or others.
  7. Explain the potential consequences of requesting, sending, or digitally posting sexually explicit pictures or messages and demonstrate the ability to make health-promoting decisions related to safe and legal activity in online and digital spaces.
  8. Demonstrate strategies for protecting privacy and reducing risks online and in digital spaces.
  9. Describe the characteristics of various forms of power and control and demonstrate strategies for getting help.
  10. Evaluate various non-violent responses to address conflict and demonstrate the ability to use these responses to act on health-related decisions.
  11. Analyze barriers that may prevent someone from reporting unsafe situations and child maltreatment to adults and identify strategies to overcome these barriers.
  12. Demonstrate the ability to use a decision-making process to address personal safety in various situations.

### *Healthy Relationships [8.4.HR]*

(Note: This section should not include anything that falls under sexual relationships or sex education. Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.)

1. Analyze the similarities and differences between friendships and romantic relationships and discuss various ways to show affection within different relationships.
2. Compare and contrast the continuum of healthier and less healthy relationship behaviors and how these affect health and well-being.
3. Identify warning signs of potential danger in a relationship and strategies for help.
4. Describe characteristics of unhealthy and/or abusive relationships

- and evaluate options and strategies a person might use to end those relationships, including involving a trusted adult who can help.
5. Define consent and describe factors, including drugs and alcohol, that may influence one's capacity to request consent, and to give and receive consent, in a variety of situations.
  6. Demonstrate the ability to apply a decision-making process to decisions related to consent in various relationships.
  7. Demonstrate the ability to apply a decision-making model to arrive at a decision that promotes health and safety related to various situations.

### *Balanced Eating and Physical Activity [8.4.BEPA]*

1. Seek strategies to minimize barriers and maximize opportunities (e.g., time, space, physical abilities, access to equipment, overuse of digital devices) that help maintain a physically active lifestyle.

### *Substance Use and Misuse [8.4.SU]*

1. Apply a decision-making process to situations involving legal or illegal drugs, considering potential consequences on self and others.
2. Determine when situations or problems related to illegal drugs might require assistance and identify possible options for how to respond.
3. Access valid and reliable information (from home, school, and community) about legal and illegal drugs (regarding age of legal use or type of substance) and use it to understand and communicate the risks and dangers of drug use and misuse.
4. Analyze factors that may affect a decision to use legal or illegal substances.
5. Use adolescent data to reinforce the norms that most don't use illegal drugs, including nicotine, alcohol, and marijuana.

## **Practice 5: Self-Management and Goal Setting**

*Set goals, engage in health-promoting behaviors, and avoid risky behaviors.*

### *Mental and Emotional Health [8.5.MEH]*

1. Explain environmental and contextual factors that affect mental and emotional well-being and individual and collective responsibility for supporting mental and emotional health.
2. Demonstrate strategies to manage changing emotions during adolescence effectively.
3. Demonstrate techniques to independently manage emotions on one's own in a variety of settings.
4. Demonstrate behaviors that will maintain or improve the mental and emotional well-being of self and others.
5. Explain possible outcomes of expressing or repressing emotions.

6. Examine how various coping strategies may help or harm health.
7. Apply health-promoting coping and stress management strategies.
8. Demonstrate strategies to persevere when facing adversity.
9. Create and monitor personal goals to meet identified emotional and mental health needs or wants and identify people or resources to assist in meeting those goals.
10. Analyze and demonstrate strategies for planning, prioritizing, and managing time.

### *Safety [8.5.SAF]*

1. Demonstrate proficiency in cardiopulmonary resuscitation (CPR) and the use of automated external defibrillators (AEDs).
2. Demonstrate ways to promote personal safety and reduce the risk of injuries in a variety of situations (e.g., during physical activity, around motor vehicles, around firearms, around loud noise or music, around water, fire prevention, during a fire, as a pedestrian).

### *Balanced Eating and Physical Activity [8.5.BEPA]*

1. Describe how self-expression and enjoyment influence individual engagement in physical activity.
2. Recognize and implement safe and appropriate behaviors during physical activity (e.g., knowing where you are, being aware of your surroundings, staying off cell phones).
3. Explain the connections between being physically active and overall physical, emotional, and mental health.
4. Apply knowledge of personal comforts and preferences to select physical activities of interest.

## **Practice 6: Advocacy and Health Promotion**

*Promote personal, family, and community health and well-being.*

### *Mental and Emotional Health [8.6.MEH]*

1. Analyze how stereotyping, bias, prejudice, and discrimination can affect mental and emotional health.
2. Analyze influences on body image and the relationship between body image, disordered eating, and mental health.
3. Analyze the effects of social media on mental and emotional health.
4. Demonstrate strategies for supporting healthy body image in youth and adolescents.
5. Demonstrate strategies for reducing stigma related to mental health.
6. Use adolescent data regarding peer norms to formulate a health-promoting position to challenge negative norms, discrimination, and injustice.
7. Evaluate strategies for countering, reducing, or eliminating prejudice, stereotyping, discrimination, and injustice.

8. Encourage others to refrain from teasing or bullying others based on personal characteristics (e.g., race, national origin, disability, body shape, or weight), or personal values and beliefs.

### *Safety [8.6.SAF]*

1. State a position, supported by accurate information, that encourages peers to adopt or continue practices that maintain or enhance personal safety.
2. Demonstrate how to influence and support others to make choices that maintain or enhance personal safety.
3. Work cooperatively to support the safety of individuals, families, and communities.
4. Identify reasons that harassment is harmful and illegal, along with warning signs for when to report it and seek help from a safe/trusted adult or health professional.

### *Balanced Eating and Physical Activity [8.6.BEPA]*

1. Demonstrate the importance of food and how it is used in cultural traditions, celebrations, and connecting with others.
2. Analyze a variety of influences (e.g., media, peers, family, culture, stigmas, school, economics, food access, food production, and cultivation) on nutrition-related beliefs and behaviors in today's society.
3. Discuss a variety of perspectives on health-promoting eating practices (e.g., food guidelines from other countries and cultures, cultural food practices, buying locally grown or produced foods) and describe the health benefits of and strategies for implementing these practices.
4. Articulate a health-promoting position on a nutrition-related topic and support the claim with accurate information.
5. Identify personal dietary needs (e.g., food allergies, food preferences) and goals, and advocate to address the identified needs and goals.
6. Collaborate effectively to support nutrition-related practices or behaviors that maintain or enhance health.

## Grade Span: 9-12 (by the end of Grade 12)

### **Practice 1: Self-Awareness and Analyzing Influences**

*Examine how emotions, thoughts, needs, values, beliefs, and other factors (both internal and external) influence behaviors and articulate how these influences affect health behavior and outcomes.*

#### *Safety [12.1.SAF]*

1. Discuss the role of personal, community, and societal beliefs, values, and actions in creating a culture free of bullying, harassment, and abuse.

#### *Healthy Relationships [12.1.HR]*

(Note: This section should not include anything that falls under sexual relationships or sex education. Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.)

1. Demonstrate how to access valid and reliable information and resources to help maintain positive relationships and get help if in harmful or unhealthy relationships.

#### *Balanced Eating and Physical Activity [12.1.BEPA]*

1. Analyze the role of technology and social media tools in supporting healthy eating and active lifestyles.
2. Analyze the impact of a variety of factors (e.g., culture, life choices, economics, motivation, and accessibility) on people's participation in physical activity.

#### *Community and Environmental Health [12.1.CEH]*

1. Explore the impact of social drivers of health (e.g., education, social environment, socioeconomic conditions, public safety) on individuals at different levels (e.g., interpersonal, intrapersonal, community, policy).
2. Evaluate the influence of social context/environment, not solely personal choices, on an individual's health.
3. Discuss various factors that can influence public, community, and/or environmental health and analyze strategies for mitigating harm and improving health outcomes.
4. Analyze the behavioral (e.g., sedentary lifestyle, smoking, dietary habits) and environmental factors (e.g., policies, access and availability, built environment) that contribute to major chronic diseases (e.g., diabetes, heart disease, lung cancer).
5. Identify and evaluate global influences (e.g., pollution, global policies) on personal and community health.
6. Analyze behaviors, policies, and practices in the school community

that promote dignity and respect and reduce stigma for all individuals.

## **Practice 2: Social Awareness, Relationship, and Communication Skills**

*Enhance relationships, personal health, and the health of others through social awareness and effective communication.*

### *Safety [12.2 SAF]*

1. Evaluate potential risks of digital interactions.
2. Discuss strategies to protect personal information online and on social media.
3. Apply safe behaviors to promote privacy, well-being, and respectful online communication.

### *Mental and Emotional Health [12.2.MEH]*

1. Describe positive (e.g., developmental assets, protective factors, resilience, supportive adult relationships) and negative factors (e.g., trauma, adversity) that can affect mental and emotional health and well-being.
2. Demonstrate strategies for expressing understanding towards those who hold different beliefs.
3. Evaluate how society, cultural norms, and values affect personal interactions.
4. Evaluate the influence of peers, social media, online content, family, society, community, and culture on body image and the influence body image has on health.
5. Evaluate personal comfort with engagement in social situations and create a plan for personal growth in social engagement.
6. Analyze power imbalances in relationships and demonstrate strategies that communicate your values, your right to say no, and your ability to hold others accountable for their actions.
7. Demonstrate the ability to communicate about mental health in culturally responsive ways that reduce stigma.
8. Communicate when there is a concern about one's own or someone else's mental well-being or when someone is considering self-harm or suicide.

### *Healthy Relationships [12.2.HR]*

(Note: This section should not include anything that falls under sexual relationships or sex education. Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.)

1. Demonstrate ways to express understanding of and acceptance of differing perspectives.



2. Use effective strategies (e.g., boundary setting, maintaining personal boundaries, respecting others' boundaries, I-statements, stating your needs, recognizing warning signs) to leave negative relationships and improve or maintain positive relationships.
3. Summarize the benefits of respecting individual differences.
4. Analyze how various factors and behaviors influence conflict and strategies to avoid escalation.
5. Demonstrate effective ways to communicate with trusted adults about bullying, harassment, abuse, assault, discrimination, or exploitation.
6. Appropriately resolve interpersonal conflicts in various settings (e.g., school, family, work, community, and personal relationships).
7. Demonstrate empathy (e.g., active listening, withholding judgment, compassion) toward others.
8. Demonstrate communication skills that account for the perspective of others while allowing for active and effective social engagement.
9. Evaluate verbal, physical, and non-verbal social, cultural, and environmental cues to predict and respond to the emotions and communication style of others.
10. Use assertive communication techniques in various settings and with a variety of audiences to meet personal needs and maintain or enhance overall health.
11. Summarize the importance of talking with parents, guardians, or other trusted adults about issues related to relationships.

### *Balanced Eating and Physical Activity [12.2.BEPA]*

1. Evaluate the opportunity for social interaction and social support in a variety of physical activities in and out of school.
2. Analyze the impact of extreme or fad diets and how social influences can encourage unhealthy eating behaviors.

## **Practice 3: Information and Resource Seeking**

*Access, evaluate, and use valid and reliable health information, products, services, and related resources.*

### *Mental and Emotional Health [12.3.MEH]*

1. Demonstrate how to access valid and reliable mental health information and resources to help or support someone who has experienced harassment, abuse, assault, discrimination, and/or exploitation.
2. Discuss the variety of supports available at home, school, and in the community for maintaining or enhancing mental and emotional health.
3. Evaluate the valid and reliable resources from home, school, and



- community that provide health information on enhancing mental and emotional well-being.
4. Use valid and reliable resources to find information and access support on mental health issues and conditions for self or others.
  5. Recognize the signs of problem behaviors and/or addictions (e.g., gambling, overuse of social media) and demonstrate strategies for seeking help for self or others.
  6. Use valid and reliable resources to find information on risk factors for, and signs and symptoms of, suicide ideation or non-suicidal self-injury.

### *Healthy Relationships [12.3.HR]*

(Note: This section should not include anything that falls under sexual relationships or sex education. Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.)

1. Explain the harm that can be caused by disrespecting others with differing views and beliefs, and demonstrate positive ways to express understanding of differing perspectives.
2. Use effective strategies (e.g., boundary setting, maintaining personal boundaries, respecting others' boundaries, I-statements stating your needs, recognizing warning signs) to leave negative relationships and improve or maintain positive relationships.
3. Summarize the benefits of respecting individual differences.
4. Reflect on the role individual behaviors and external factors have in a conflict and discuss how individual behaviors and external factors may inform the ability to resolve conflict in the future.
5. Demonstrate effective ways to communicate with trusted adults about bullying, harassment, abuse, assault, discrimination, or exploitation.
6. Appropriately resolve interpersonal conflicts in various settings (e.g., school, family, work, community, and personal relationships).
7. Demonstrate empathy (e.g., active listening, withholding judgment, compassion) toward others.
8. Demonstrate communication skills that account for the perspective of others while allowing for active and effective social engagement.
9. Evaluate verbal, physical, and non-verbal social, cultural, and environmental cues to predict and respond to the emotions and communication style of others.
10. Use assertive communication techniques, including refusals, in a variety of settings and with a variety of audiences to meet personal needs and maintain or enhance overall health.
11. Summarize the importance of talking with parents, guardians, caregivers, and/or other trusted adults about issues related to relationships.

### *Balanced Eating and Physical Activity [12.3.BEPA]*

1. Evaluate the validity of claims made by companies and social media influencers intended to promote or improve nutrition, fitness, and a healthy, active lifestyle.
2. Evaluate available resources, supports, and participation requirements of community-sponsored activities, physical activity, and fitness activities.
3. Evaluate the opportunity for social interaction and social support in a variety of physical activities in and out of school.

### *Substance Use and Misuse [12.3.SU]*

1. Evaluate the validity of information, products, services, and resources intended to help a person make health-promoting choices when making decisions related to legal substance use.
2. Use valid and reliable information to analyze the relationship between the use of legal and illegal (regarding legal age of use and type) substances and the causes of death (including overdose) and disease (including addiction) in the United States.
3. Recognize the signs of substance misuse and addiction and demonstrate strategies for seeking help for self or others.
4. Identify resources and support systems available to help navigate challenging situations pertaining to substance use and misuse.
5. Identify laws that protect a person who calls for professional help during a substance use crisis (e.g., Good Samaritan laws).
6. Access valid and reliable resources to determine laws regarding the purchase, distribution, and selling of substances (e.g., dispensaries, on the street, stores, legal vs. illegal, travel across state lines).
7. Determine when professional services related to legal and illegal substances may be required.

### *Community and Environmental Health [12.3.CEH]*

1. Identify cost-effective ways to minimize environmental pollutants (e.g., chemicals, trash, noise) in the home and in the community.

## **Practice 4: Decision Making and Problem Solving**

*Make health-promoting, informed, responsible decisions and solve problems in a variety of health-related situations.*

### *Healthy Relationships [12.4.HR]*

(Note: This section should not include anything that falls under sexual relationships or sex education. Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.)

1. Examine aspects of various personal relationships, including

- characteristics of healthy and unhealthy relationships.
2. Analyze the benefits and risks of various ways people express love and/or caring within health-promoting relationships.
  3. Analyze the role of individual versus shared responsibility in building and maintaining healthy relationships.
  4. Describe patterns of power and control in relationships and discuss strategies for getting help and leaving an unhealthy, violent, or exploitative relationship.
  5. Discuss different forms of abuse in relationships and available supports and resources for getting help if in an abusive or exploitative relationship.
  6. Evaluate strategies (e.g., setting expectations and/or boundaries) for dealing with difficult relationships with family members, peers, and partners and demonstrate the ability to use these strategies to make health-promoting decisions.
  7. Examine the complexity of, and discuss considerations related to, the decision to leave an unhealthy relationship, develop a safety plan to recognize when, and get out of, any future unsafe or harmful relationships, and determine situations when adult and/or professional support is needed.
  8. Demonstrate the ability to apply a thoughtful decision-making process to maintain or enhance relationship health, including the decision to leave or seek help in an unhealthy relationship.

### *Balanced Eating and Physical Activity [12.4.BEPA]*

1. Identify snacks, food, and beverage choices that support or interfere with performance, recovery, and enjoyment during physical activity.
2. Plan a weekly menu to meet age-appropriate recommendations within a specific budget.
3. Choose an appropriate level of challenge based on your current skill level to experience success in a self-selected physical activity.
4. Evaluate opportunities and barriers to physical activity in a variety of contexts.

### *Substance Use and Misuse [12.4.SU]*

1. Reflect on personal beliefs, values, and choices compared to cultural, community, and societal norms around substance use and misuse.
2. Evaluate situations and how various internal and external factors (e.g., peers, media, social norms, corporate practices) influence substance use and misuse.
3. Analyze the potential short- and long-term impacts of legal and illegal substances (e.g., opioids) on multiple dimensions of health

- (e.g., physical, mental, emotional, social, and/or intellectual) and on other health risk behaviors (e.g., sexual activity, impaired driving, use of other drugs).
4. Employ self-management skills to act on health-promoting decisions about legal substance use.

## **Practice 5: Self-Management and Goal Setting**

*Set goals, engage in health-promoting behaviors, and avoid risky behaviors.*

### ***Mental and Emotional Health [12.5.MEH]***

1. Apply strategies to respond appropriately based on different levels of emotions in interactions.
2. Apply coping and stress management techniques to manage a variety of stressors (e.g., school, personal life, relationships) and create a long-term plan for stress management.
3. Develop and apply strategies using protective factors and assets to support mental and emotional well-being.
4. Assess and implement health practices and overall health status across multiple dimensions of wellness (e.g., physical, emotional, intellectual, spiritual, social).
5. Set a goal, create a plan, monitor progress, and celebrate success for plans that minimize stress and promote wellness.
6. Apply strategies that support a willingness to reflect, learn, and grow from challenges through experience and feedback.

### ***Safety [12.5.SAF]***

1. Analyze recruitment tactics used in trafficking and exploitation to exploit vulnerabilities and recruit youth.
2. Demonstrate strategies, including risk reduction strategies, that can help avoid or address situations related to sexual exploitation in physical and digital settings.
3. Evaluate ways and demonstrate strategies to reduce risk and stay safe, follow laws, and act respectfully in physical and digital settings.
4. Demonstrate strategies for asking for assistance or providing support for self and peers when faced with unsafe situations.
5. Demonstrate proficiency in cardiopulmonary resuscitation (CPR) and the use of automated external defibrillators (AEDs).

### ***Balanced Eating and Physical Activity [12.5.BEPA]***

1. Compare and contrast various dietary guidelines and practices from valid and reliable sources, locations, and cultures.
2. Analyze the physical, mental, social, economic, and academic benefits and/or consequences of various dietary habits or

- behaviors.
3. Describe safe food storage and preparation practices.
  4. Demonstrate how to comparison shop, considering criteria such as pricing, nutrient density, processing, and environmental impact.
  5. Assess personal nutrition-related practices using dietary guidelines of their choice.
  6. Analyze various factors that influence nutrition-related beliefs and behaviors, and analyze the ways that these factors are affecting personal beliefs and behaviors.
  7. Develop and implement a plan with goals that build on strengths and address areas for improvement through monitoring progress and adjustments as needed.
  8. Analyze barriers (e.g., finances, food availability and access, social norms, media) to succeed with a personal nutrition-related goal and identify supports to help overcome those barriers.
  9. Determine a variety of physical activities that can be implemented independently or with minimal support for the purposes of personal enjoyment or challenge, or to maintain or improve fitness.
  10. Create a goal and plan for participating in physical activity to maintain or improve health and monitor progress.
  11. Select and participate in physical activities that meet a variety of personal needs (personal goals, strengths, interests, enjoyment, social interaction, and/or self-expression).

### *Personal Health and Wellness [12.5.PHW]*

1. Develop a plan and implement strategies based on an identified need or want to attain a goal that improves physical health.
2. Create a plan that develops ownership of one's health and healthcare.
3. Demonstrate strategies to self-advocate in healthcare settings (e.g., getting questions answered, seeking clarity of medical instructions, seeking a prescription refill).
4. Identify strategies based on an identified need or want to attain a goal that improves physical health.
5. Demonstrate strategies to self-advocate in healthcare settings (e.g., getting questions answered, seeking clarity of medical instructions, seeking a prescription refill).
6. Discuss signs and symptoms of health concerns and strategies to manage discomfort and/or seek medical care as needed.
7. Analyze external factors (such as social drivers of health) that can affect health and one's agency in addressing health.

## **Practice 6: Advocacy and Health Promotion**

*Promote personal, family, and community health and well-being.*

### *Community and Environmental Health [12.6.CEH]*

1. Analyze programs, policies, and strategies to reduce and eliminate health inequities and disparities.
2. Examine data and evaluate policies or initiatives that address a public health concern within the community.
3. Develop and/or implement a plan for participating in projects to help make positive changes in a community (e.g., volunteering, linking with experts in the community, service-learning, service project).
4. Identify and support school and community policies and programs that promote respect for all people.



## SECTION 3. LAWS AND STANDARDS GUIDELINES SPECIFIC TO SEX EDUCATION

## Section 3 Introduction: Michigan Laws Specific to Sex Education Instruction

Listed below are laws that center around educator qualifications, districts' responsibilities regarding sex education instruction, parent rights regarding sex education instruction, and requirements of what needs to be taught if a district chooses to include sex education as part of its health education program.

### Educator Qualifications for Sex Education Instruction

A teacher with an elementary certificate (grades K-5) with an endorsement that includes "all subjects" is qualified to teach health education for the grades designated. A teacher with a secondary certificate (grades 6-12) is authorized to teach health education with any one of the following endorsements: MC (Health and Physical Education), *MA (health)*, *MC (K-12 Health and Physical Education)*, *MX (health, physical education, recreation, and dance)*, and KH (family and consumer science). Please note that the italicized endorsements are no longer issued and are in the process of being phased out. For more information regarding whether your certification allows you to teach health, please go to the Michigan Department of Education's [Quick Reference Guide: Courses That Can Be Taught page](#).

Although the law allows teachers to teach sex education if they have the endorsements listed above, it is highly recommended by the Michigan Department of Education that teachers receive professional development in sex education instruction, which includes training in curriculum implementation, best practice, and current state law related to sex education. Please contact your [Regional School Health Coordinator](#) to inquire about current training opportunities.

The Michigan Department of Education clarifies the qualifications of a person who may provide sex education instruction:

- Consistent with [MCL 380.1169](#) that mandates HIV/AIDS instruction, any certified teacher shall be qualified to provide such instruction upon the successful completion of an in-service program provided by designated [Regional School Health Coordinators](#) and approved by the Michigan Department of Education.
- Apart from certified teachers, [MCL 380.1169\(2\)](#) also allows licensed health care professionals who have training in HIV/AIDS to provide this instruction.

### Local Control and Parent Choice in Sex Education Instruction

Local education agencies (LEAs), including intermediate school districts,



public school academies, and traditional public school districts, have local control in determining whether or not they include sex education in their health education course. If they choose to include sex education, LEAs then determine what content within sexual health instruction they will include in their health education program. LEAs are encouraged to use student and community data, parent input, and community values to determine what content will best support the health and well-being of students in their district.

Michigan law has some of the strongest requirements in the country for parental involvement and oversight in sex education. If a district chooses to include sex education as part of its comprehensive health education program, the district must establish a sex education advisory board (SEAB). This board must consist of at least 50% parent membership. The majority of the parents cannot be currently employed by the school district. The SEAB must also include at least one parent as a co-chair. SEABs review student data, review curricula, and make recommendations to the local district's board of education for review and approval. The SEAB is responsible for:

- Establishing program goals and objectives that are likely to reduce the rates of sex, pregnancy, and sexually transmitted infections.
- Reviewing materials and methods of instruction and making recommendations to the board of the school district.
- Evaluating, measuring, and reporting on the attainment of goals at least every two years. [[MCL 380.1507\(5\)](#)]

By law ([MCL 380.1507](#)), parents shall:

- Receive prior notification of sex education classes and curriculum.
- Have the right to review sex education curriculum prior to instruction.
- Be able to opt out their child from all or some of the sex education content without penalty or loss of academic credit.

The parent rights listed above also pertain to HIV instruction under [MCL 380.1169](#), which is covered in Section 1 of this document.

## Required Sex Education Content Should Districts Instruct in Sex Education

If schools choose to include a sex education unit in their health education course, there are laws regarding guidelines for instruction:

- Sex education instruction shall emphasize that abstinence from sex is the only protection that is 100% effective against unplanned pregnancy, sexually transmitted infections, and HIV ([MCL 380.1507b](#)).
- Material and instruction shall be age-appropriate and medically

accurate and shall do at least the following ([MCL 380.1507b](#)):

- Discuss benefits from abstaining from sex and the benefits of ceasing sex if a student is already sexually active
- Include a discussion of the possible emotional, economic, and legal consequences of sex
- Stress that unplanned pregnancy and sexually transmitted diseases are serious possibilities of sex that are not fully preventable except by abstinence
- Advise pupils of the laws pertaining to their responsibility as parents
- Ensure that pupils are not taught in a way that condones the violation of the laws of this state pertaining to sexual activity
- Teach pupils how to say "no" to sexual advances and that it is wrong to take advantage of, harass, or exploit another person sexually
- Teach refusal skills and encourage pupils to resist pressure to engage in risky behavior
- Teach that the pupil has the power to control personal behavior.
- Provide instruction on healthy dating relationships and on how to set limits and recognize a dangerous environment
- Provide information for pupils about how young parents can learn more about adoption services and about the provisions of the safe delivery of newborns law
- Include information clearly informing pupils that having sex or sexual contact with an individual under the age of 16 is a crime punishable by imprisonment and that one of the other results of being convicted of this crime is to be listed on the sex offender registry
- Clinical abortion shall not be considered a method of family planning or be taught as a method of reproductive health ([MCL 380.1507](#)).
- A person shall not dispense or otherwise distribute in a public school or on public school property a family planning drug or device ([MCL 380.1507](#)).
- This section does not prohibit a public school from offering sex education with behavioral risk reduction strategies, as defined by law, that are not 100% effective against unplanned pregnancy, sexually transmitted disease, and sexually transmitted human immunodeficiency virus infection and acquired immunodeficiency syndrome ([MCL 380.1507b](#)).

## Michigan Elliott-Larsen Civil Rights Act

Teaching students the definitions surrounding gender identity and diversity aligns with the Michigan [Elliott-Larsen Civil Rights Act](#) (Public Act 6 of

2023), which includes provisions that ensure equal opportunities for a quality education and promotes inclusivity, which includes sexual orientation and gender identity or expression as protected categories.

It is important for local districts to remember that if they include lessons that aim to cover the standards guidelines specific to gender diversity, those lessons and materials must be medically accurate and age-appropriate. The inclusion of the lessons also must follow all laws outlined here, including parent opt-out provisions.

The research is clear that when such topics as gender identity are presented in a way that is medically accurate and developmentally appropriate, children experience lower rates of bullying, harassment, and suicide. (American College of Obstetricians and Gynecologists; American Academy of Pediatrics) If we want children to excel in the basics of reading, writing, and math, we must provide environments where all students feel safe (American Academy of Pediatrics, U.S. Department of Education; Education Sciences)

Gender-diverse students often report feeling unsafe at school and experience greater rates of harassment. They indicate that they rarely report discriminatory incidents because when they do report them, they feel unprotected. Gender-diverse students experience greater gender-related stress at school and are more likely to be absent, have lower GPAs, report higher levels of depression, engage in more substance use and risky behaviors, and are at an elevated risk for suicide.

Michigan's gender diverse students are:

- 1.8 times more likely to have been threatened or injured with a weapon at school.
- 2.1 times more likely to skip school because they feel unsafe.
- 1.6 times more likely to have been bullied at school or online.

It's important to note that gender-diverse students are not inherently prone to mental and physical health risks because of their sexual orientation or gender identity. They are at higher risk due to experiences of discrimination, stigmatization, and victimization in society.

## Section 3: Sex Education Standards Guidelines

### Grade Span: K-2 (by the end of Grade 2)

Purposely Blank

There are NO standards guidelines in this grade span.

## Grade Span: 3-5 (by the end of Grade 5)

### **Practice 3: Information and Resource Seeking**

*Access, evaluate, and use valid and reliable health information, products, services, and related resources.*

#### *Sex Education [5.3.SE]*

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Identify valid and reliable information, products (deodorant, period products, medicine for cramps, etc.), and resources related to growth and development, puberty, and personal hygiene.
2. Locate resources from home, school, and community that provide medically accurate sources of information about puberty, personal hygiene, and growth and development.
3. Use valid, reliable, and medically accurate resources to find information about the human reproductive systems, growth and development, and the effects of hormones.
4. Explain human reproduction and identify valid and reliable resources for additional information.
5. Identify parents, guardians, or other trusted adults (e.g., counselors and other health care professionals) whom students can ask questions about puberty, abstinence, and adolescent health issues (including abuse and neglect).
6. Define communicable diseases, including Human Immunodeficiency Virus (HIV), and identify how they are and are not transmitted.

### **Practice 5: Self-Management and Goal Setting**

*Set goals, engage in health-promoting behaviors, and avoid risky behaviors.*

#### *Sex Education [5.5.SE]*

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Describe the range of physical, social, and emotional changes during puberty and adolescence and the individual variations in puberty timelines and experiences.
2. Explain various health-promoting practices to manage the social, physical, and emotional changes associated with puberty and adolescence, as well as when help or support might be needed.
3. Describe health-promoting behaviors during menstruation, including ways to cope with emotional changes, manage pain, and identify when help or support is needed.

4. Practice healthy habits related to puberty and personal hygiene.

## Grade Span: 6-8 (by the end of Grade 8)

### **Practice 2: Social Awareness, Relationship, and Communication Skills**

#### *Sex Education [8.2.SE]*

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Define gender identity, gender expression, and sexual orientation, and explain that they are distinct components of every individual's identity.
2. Explain how biological sex, gender identity, and gender expression are distinct concepts and how they interact with each other.
3. Explain that romantic, emotional, and/or sexual attractions can be toward an individual of the same and/or different gender(s), and that attractions can change over time.
4. Discuss signs, symptoms, and potential effects of sexually transmitted infections, including HIV.

### **Practice 3: Information and Resource Seeking**

*Access, evaluate, and use valid and reliable health information, products, services, and related resources.*

#### *Sex Education [8.3.SE]*

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Analyze the validity of claims for health information, products (e.g., period products, personal hygiene, over-the-counter pain medications), services, and resources about sexual and reproductive health.
2. Access credible sources of information about sexual and reproductive health.
3. Locate valid and reliable information on puberty, personal hygiene, menstruation, and personal health products from various resources in one's home, school, and community.

### **Practice 4: Decision Making and Problem Solving**

*Make health-promoting, informed, responsible decisions and solve problems in a variety of health-related situations.*

#### *Sex Education [8.4.SE]*

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Analyze personal and family values related to relationships, abstinence, sexual behaviors, and sexual health.
2. Identify valid and reliable sources of information and resources to inform and support sexual health decisions, including abstinence.
3. Analyze ways to prevent pregnancy and sexually transmitted infections (STIs), including strategies that can be used before becoming sexually active (e.g., abstinence, communicating with a partner, HPV vaccine, contraception).
4. Describe possible short- and long-term impacts of engaging in sexual activity and identify ways to avoid negative or potentially harmful consequences.
5. Explain the importance of, and ways to identify, setting personal limits to avoid unintended outcomes from risky or unwanted sexual behavior and to make sexual health decisions.
6. Articulate the benefits of abstinence, postponing sexual activity, and setting personal limits (e.g., aligning with personal or family values, understanding the changing nature of relationships, avoiding early or unintended pregnancy, reducing risk of STIs) based on individual beliefs and values.
7. Describe strategies that can be used to make decisions that adhere to personal and family values.
8. Apply an effective decision-making process in situations related to sexual health.

## **Practice 5: Self-Management and Goal Setting**

*Set goals, engage in health-promoting behaviors, and avoid risky behaviors.*

### **Sex Education [8.5.SE]**

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Examine various considerations (e.g., personal and/or family values, cultural and societal norms, and beliefs) for determining emotional readiness for sexual behaviors.
2. Assess personal health practices and develop short- and long-term goals that support healthy sexual behaviors (e.g., abstinence, delay, use of contraception, use of barriers, giving and obtaining consent).
3. Determine strategies, including abstinence, that will reduce the risk of HIV and other sexually transmitted infections and pregnancy.
4. Describe how sexual health values and priorities may change with age, maturity, knowledge, and responsibilities.



## **Practice 6: Advocacy and Health Promotion**

*Promote personal, family, and community health and well-being.*

### ***Sex Education [8.6.SE]***

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Demonstrate ways to show courtesy and respect for others when aspects of their sexuality or gender are different from one's own.
2. Practice skills to intervene if teasing or bullying based on sexuality is occurring, and how to support those affected.
3. Encourage others to refrain from teasing or bullying others based on their sexuality (e.g., sexual activity [including abstinence], sexual orientation) or gender (e.g., gender expression, gender identity).
4. Identify behaviors, policies, and practices in the school community that promote or hinder dignity and respect for all individuals.

## Grade Span: 9-12 (by the end of Grade 12)

### **Practice 1: Self-Awareness and Analyzing Influences**

*Examine how emotions, thoughts, needs, values, beliefs, and other factors (both internal and external) influence behaviors and articulate how these influences affect health behavior and outcomes.*

#### **Sex Education [12.1.SE]**

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Analyze a variety of internal and external influences (e.g., family, peers, media, society, community, culture) on a person's attitudes, beliefs, and expectations about abstinence and sexual behavior.

### **Practice 2: Social Awareness, Relationship, and Communication Skills**

*Enhance relationships, personal health, and the health of others through social awareness and effective communication.*

#### **Sex Education [12.2.SE]**

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Summarize the importance of talking with parents, guardians, or other trusted adults about issues related to growth and development, abstinence, and sexual health.

### **Practice 3: Information and Resource Seeking**

*Access, evaluate, and use valid and reliable health information, products, services, and related resources.*

#### **Sex Education [12.3.SE]**

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Summarize the benefits of respecting individual differences in aspects of growth and development
2. Summarize the importance of talking with parents, guardians, or other trusted adults about issues related to growth and development and sexual health.

### **Practice 4: Decision Making and Problem Solving**

*Make health-promoting, informed, responsible decisions and solve problems in a variety of health-related situations.*

### ***Sex Education [12.4.SE]***

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Discuss reasons why it is harmful and illegal to trick, threaten, or coerce another person into sexual activity.
2. Analyze factors that contribute to behaviors that increase the risk of pregnancy, HIV, and other STIs.
3. Explain the importance of STI (including HIV) testing and treatment, where to get tested, and why it is essential to communicate with a partner about STI status.
4. Identify situations, signs, and symptoms that might indicate a need to seek medical consultation.
5. Evaluate readiness, options, and their respective outcomes regarding decisions about whether to engage in sexual activity, including abstaining, postponing sexual intercourse, engaging in risk reduction practices (e.g., using condoms and other barriers, using birth control).
6. Demonstrate the ability to apply a thoughtful decision-making process in situations related to sexual activity and sexual health.
7. Explain age of consent laws and examine the various components of consent (e.g., consent must be asked for and verbally given, consent cannot be given if under the influence, consent can be taken away at any time).

### **Practice 5: Self-Management and Goal Setting**

*Set goals, engage in health-promoting behaviors, and avoid risky behaviors.*

#### ***Sex Education [12.5.SE]***

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Evaluate personal responsibility and the consequences related to pressuring someone for sexually explicit pictures, sending, or posting sexually explicit pictures or messages.

### **Practice 6: Advocacy and Health Promotion**

*Promote personal, family, and community health and well-being.*

#### ***Sex Education [12.6.SE]***

Sexual relationship topics fall under sex education in these standards guidelines and must follow state laws on sex education instruction.

1. Discuss how to foster empathy, inclusivity, and respect around issues related to gender and sexuality.



## APPENDICES. MICHIGAN LAWS GOVERNING HEALTH AND SEX EDUCATION

## Appendix A

### Michigan Laws Related to Health Education, Including Sex Education

[MCL 380.1169](#) Dangerous communicable diseases; human immunodeficiency virus infection and acquired immunodeficiency virus infection; teacher training; teaching materials; curricula; teaching of abstinence from sex.

[MCL 380.1137](#) Powers of parents and legal guardians; policies or guidelines.

[MCL 380.1170](#) Physiology and hygiene; instruction; development of comprehensive health education programs; conflict with religious beliefs.

[MCL 380.1170a](#) Model core academic curriculum content standards guidelines for health education; subject area content expectations and guidelines for health education; instruction in cardiopulmonary resuscitation and automated external defibrillators; individuals providing instruction; use of local resources; exemption; definitions.

[MCL 380.1170b](#) State model academic standards guidelines for health education; inclusion of instruction on prescription opioid drug abuse; availability.

[MCL 380.1502](#) Health and physical education; establishment; course in physical education required; extracurricular athletics as meeting requirement.

[MCL 380.1506](#) Program of instruction in reproductive health; supervision; request to excuse pupil from attendance; "reproductive health" defined.

[MCL 380.1507](#) Instruction in sex education; instructors, facilities, and equipment; stressing abstinence from sex; elective class; notice to parent or guardian; request to excuse pupil from attendance; qualifications of teacher; sex education advisory board; public hearing; distribution of family planning drug or device prohibited; "family planning," "class," and "course" defined.

[MCL 380.1507a](#) Notice of excuse from class; enrollment.

[MCL 380.1507b](#) Sex education and instruction; curriculum requirements.

[Act 453 of 1976](#) Elliott-Larsen Civil Rights Act

## Appendix B

### **Michigan Laws that Provide Recommendations for Health Education Programming**

[MCL 380.1502](#) Health and physical education; establishment; course in physical education required; extracurricular athletics as meeting requirement.

[MCL 380.1310b](#) Policy prohibiting bullying; adoption and implementation; public hearing; submission of policy to department; contents of policy; annual report of incidents of bullying; form and procedure; school employee, school volunteer, pupil, or parent or guardian reporting act of bullying to school official; modified policy; definitions; section to be known as "Matt Epling Safe School Law."

[MCL 380.1505](#) Sexual abuse of children; adoption and implementation of policy.

[MCL 380.1505a](#) Instruction to students on child sexual abuse

[MCL 380.1171](#) Suicide prevention and awareness; instruction and professional development; availability of model programs and materials to school districts and public school academies; notice to parents; cause of action or legal duty not created; section known as "Chase Edwards law."

[MCL 380.1170b](#) State model academic standards guidelines for health education; inclusion of instruction on prescription opioid drug abuse; availability.